

# Culturally Competent Patient Care - Factoids

Week 8 HPC • Working with Interpreters • current Spearman-McCarthy deck controls

**Core encounter rule:** use a trained interpreter, but maintain the clinician-patient relationship. Speak directly to the patient in first person.

## LEP + Language Access

### LEP

Primary language is not English + limited ability to speak, read, write, or understand English.

### Do not assume

Conversational English ≠ safe comprehension of complex medical information.

## Translation vs Interpretation

| Translation               | Interpretation                                                  |
|---------------------------|-----------------------------------------------------------------|
| Written content           | Spoken content                                                  |
| More exact; may take time | Immediate; prioritizes understanding/communication in real time |

## Why Professional Interpreters?

- More accurate/timely diagnosis
- Better access and satisfaction
- Fewer errors and adverse events
- More cost-effective diagnosis/treatment
- Improved medication/treatment adherence

**Legal / systems anchor:** current lecture links interpreters to safety, liability, Title VI, CLAS standards, and accreditation expectations. The companion explicitly identifies Title VI as the principal federal LEP language-access anchor.

## Before the Visit

| Action                                    | Reason                          |
|-------------------------------------------|---------------------------------|
| Use trained/certified medical interpreter | Accuracy + clinical safety      |
| Allow extra time                          | Most communication occurs twice |
| Minimize distractions                     | Protect attention/accuracy      |
| Brief interpreter on goals/procedures     | Prepares vocabulary and flow    |
| Document interpreter name/certification   | Medical-record accountability   |

## During the Visit

**Face the patient • first person • normal voice • short/simple language • sentence-by-sentence • watch nonverbal cues.**

- Do not address introductions/questions to the interpreter.
- Avoid jargon and idioms.
- Do not shout.
- Allow clarifying questions when needed.
- Observe before interrupting a long interpreter-patient exchange; intervene if it moves outside the interpreter role.

# INTERPRET + Wrap-Up

Mnemonic is explicit in the handout; current deck teaches the same behaviors as Before/During/Wrap-Up checklists.

| Letter | Meaning         | What to do                                                                   |
|--------|-----------------|------------------------------------------------------------------------------|
| I      | Introduction    | Introduce clinician + interpreter; establish role/confidentiality.           |
| N      | Note Goals      | Brief pre-session; state purpose/agenda.                                     |
| T      | Transparency    | Everything said is interpreted; short/simple phrases; no side conversations. |
| E      | Ethics          | Protect confidentiality, dignity, professional boundaries.                   |
| R      | Respect Beliefs | Attend to cultural values without stereotyping.                              |
| P      | Patient Focus   | <b>Talk to patient, not interpreter.</b> First person + eye contact.         |
| R      | Retain Control  | Clinician guides pace/interview and clarifies unexpected long exchanges.     |
| E      | Explain         | Use teach-back to confirm understanding.                                     |
| T      | Thank           | Thank both; handout suggests brief debrief if useful.                        |

## Wrap-Up

- **Teach-back:** patient explains directions/recommendations back in their own words.
- Leave time for questions and clarification.
- Assume side comments may be understood even if patient has LEP.
- If language assistance is declined, document that it was offered and the patient's preference.

## Faculty Vignette

**75-year-old Mandarin-speaking patient + certified interpreter:** the best technique is to **speak in simple terms and one sentence at a time.**

| Wrong move                                    | Why                                                    |
|-----------------------------------------------|--------------------------------------------------------|
| Face/speak to interpreter                     | Breaks patient focus                                   |
| Use family simply because interpreter is late | Ad hoc interpretation is not preferred for convenience |
| Speak in third person                         | Use first person                                       |
| Only closed-ended clarification               | Open-ended clarification may be needed                 |

**Last 20 seconds:** professional interpreter • direct-to-patient first person • short phrases • no jargon/shouting • sentence-by-sentence • teach-back • document.

Primary: current 30-slide Spearman-McCarthy deck. Matching AI handout is secondary, especially for the explicit INTERPRET mnemonic. Video content is not expanded because no transcript was provided.